

AFTER VISIT SUMMARY

Cascade Health Center · Comprehensive Gender Care Program
Cascade Health Center · 412 NE Irving St · Portland, OR 97212

Visit Date: 2026-05-14
Provider: Dr. Priya Mehta, MD — Comprehensive Gender Care
NPI 1234567890
(503) 555-0187

PATIENT NAME	DATE OF BIRTH	PRONOUNS	MRN	SEX ASSIGNED AT BIRTH
Jordan Reyes	1995-08-14 (Age 29)	she/her	MRN-2948371	Male

REASON FOR VISIT

Routine 6-month HRT monitoring visit. Patient presents for labs review, medication reassessment, and follow-up on prior concerns regarding mood stability and injection site reactions.

VITAL SIGNS

Blood Pressure	Pulse	Temperature	Weight	Height	BMI	SpO ₂
118 / 74 mmHg	72 bpm	98.4 °F	148 lbs (67.1 kg)	52 83 (173 in)	22.6	99%

ASSESSMENT & PLAN

1. Feminizing HRT — Stable

Estradiol levels (312 pg/mL) are within the mid-therapeutic range for feminizing HRT. Testosterone is well-suppressed at 18 ng/dL. Patient reports satisfactory feminization progress, stable mood, and no injection site complications. Continue current regimen.

- Estradiol valerate 4 mg IM every 7 days — CONTINUE
- Spironolactone 100 mg PO daily — CONTINUE with electrolyte monitoring
- Micronized progesterone 100 mg PO nightly — CONTINUE

2. Elevated Prolactin — Monitor

Prolactin at 42.1 ng/mL is mildly elevated above the standard female reference range (2.0–29.2 ng/mL). This is a common, expected finding on estrogen therapy. However, values above 30–40 ng/mL warrant monitoring. Pituitary adenoma (prolactinoma) is unlikely but should be ruled out if levels continue to rise.

- Repeat prolactin in 3 months. If >50 ng/mL, order pituitary MRI.
- No medication change at this time.

3. Elevated ALT / Borderline AST — Follow Up Required

ALT is 67 U/L (reference <56), above the upper limit of normal. AST is borderline at 38 U/L. This warrants attention — estradiol and spironolactone can both affect hepatic metabolism. Patient denies alcohol use, new supplements, or herbal products. Hepatitis B/C not indicated by history.

- Repeat LFTs in 4–6 weeks off alcohol and new supplements.
- If ALT remains >60 at recheck, consider hepatology referral and dose adjustment.
- Discussed with patient: liver enzymes can fluctuate; one elevated reading is not an emergency.

4. Potassium — Borderline High (Spironolactone Effect)

Potassium at 5.2 mEq/L is just above the upper limit of the reference range (3.5–5.1). Spironolactone is a potassium-sparing diuretic and mild hyperkalemia is an expected side effect. Patient is asymptomatic (no muscle weakness, palpitations).

- Avoid high-potassium foods (bananas, potatoes, spinach, supplements).
- Repeat BMP in 4 weeks. If K+ ≥5.5 mEq/L, reduce spironolactone to 75 mg.

5. Vitamin D Deficiency — New Supplement

25-OH Vitamin D at 28 ng/mL is mildly deficient (optimal >30 ng/mL). Initiating supplementation.

- START Vitamin D3 2000 IU PO once daily with food.
- Recheck 25-OH Vitamin D at next 6-month visit.

6. Post-Orchiectomy Care — Ongoing

Patient underwent bilateral orchiectomy on March 15, 2023 (27 months prior). Recovery complete; no ongoing surgical concerns. Bone density baseline DEXA scan overdue — ordered today.

- DEXA scan ordered — schedule within 60 days.
- Continue estrogen replacement for bone protection.

7. Mental Health Screening

PHQ-9 score: 4 (minimal depression). GAD-7 score: 3 (minimal anxiety). Patient reports overall improved quality of life since starting HRT. Noted ongoing stress related to workplace — see provider note below.

- Continue current mental health support. Referred to PFLAG support group.

PROVIDER NOTE — WORKPLACE INCIDENT

Patient disclosed a workplace discrimination incident (healthcare context) — documented at patient request.

Patient reported that on approximately May 8, 2026, a supervisor at her place of employment (Ridgeline Medical Group, Portland OR) referred to her by incorrect pronouns and her deadname during a staff meeting, despite prior written notice to HR of her legal name change. When patient requested correction, supervisor stated "I'll call you what your badge says," and the badge had not been updated per patient's formal request submitted February 2026.

Patient experienced distress and left the meeting. No physical altercation. Witnesses present: two colleagues (names withheld pending patient decision to file). Patient has preserved the original HR request email, the meeting notes, and a follow-up email to HR supervisor (Darren Kowalski) sent May 9, 2026.

Provider recommends patient consult Oregon Bureau of Labor and Industries (BOLI) and the National Center for Transgender Equality for next steps. This note is made for the medical record at patient request.

CURRENT MEDICATIONS

Medication	Dose	Route	Frequency	Since
Estradiol valerate	4 mg	IM injection	Every 7 days	Jan 2022
Spironolactone	100 mg	Oral	Once daily	Nov 2021
Micronized Progesterone	100 mg	Oral (bedtime)	Once nightly	Jun 2023
Vitamin D3 (NEW)	2000 IU	Oral	Once daily	Today

SURGICAL HISTORY

Bilateral orchiectomy · March 15, 2023 · Surgeon: Dr. A. Fontaine · Facility: Oregon Health & Science University

LABORATORY RESULTS — Collected: 2026-05-14 · Resulted: 2026-05-14

Reference ranges adjusted for feminizing HRT where applicable. H = High · L = Low. Highlighted rows indicate flagged values.

Test	Result	Unit	Reference	Flag
Hormone Panel				
Estradiol (E2)	312	pg/mL	15–350 (HRT target)	—
Testosterone, Total	18	ng/dL	<50 (feminizing HRT)	L
Testosterone, Free	1.4	pg/mL	<6.8 (female range)	—
LH	0.5	mIU/mL	1.7–8.6	L
FSH	0.8	mIU/mL	1.5–12.4	L
Prolactin Recheck in 3 months; estrogen effect likely	42.1	ng/mL	2.0–29.2	H
SHBG	87	nmol/L	18–114 (female)	—
Complete Blood Count (CBC)				
WBC	6.2	K/ μ L	4.5–11.0	—
RBC	4.1	M/ μ L	3.9–5.2 (female)	—
Hemoglobin	12.4	g/dL	11.6–15.0 (female)	—
Hematocrit	37.2	%	35–45 (female)	—
MCV	86	fL	80–100	—
Platelets	218	K/ μ L	150–400	—
Comprehensive Metabolic Panel (CMP)				

Sodium	138 mEq/L	136–145	—
Potassium	5.2 mEq/L	3.5–5.1	H
Borderline — spironolactone effect; repeat in 4 wks			
Chloride	101 mEq/L	98–107	—
CO2 (Bicarbonate)	25 mEq/L	22–29	—
BUN	11 mg/dL	7–20	—
Creatinine	0.63 mg/dL	0.50–1.10 (female)	—
eGFR	>60 mL/min/1.73m ²	>60	—
Glucose	88 mg/dL	70–99	—
Calcium	9.4 mg/dL	8.6–10.3	—
Total Protein	7.1 g/dL	6.1–8.1	—
Albumin	4.3 g/dL	3.5–5.0	—
Liver Function Tests (LFT)			
AST (SGOT)	38 U/L	10–40	—
Borderline upper limit; monitor			
ALT (SGPT)	67 U/L	7–56	H
Elevated — discuss with provider; spiro/E2 monitoring			
Alkaline Phosphatase	74 U/L	44–147	—
Total Bilirubin	0.8 mg/dL	0.2–1.2	—
Lipid Panel (fasting ×12h)			
Total Cholesterol	188 mg/dL	<200 (optimal)	—
LDL Cholesterol	108 mg/dL	<130	—
HDL Cholesterol	62 mg/dL	>50 (female)	—
Triglycerides	142 mg/dL	<150	—
Non-HDL Cholesterol	126 mg/dL	<160	—
Additional Markers			
HbA1c	5.4 %	<5.7	—
TSH	1.82 µIU/mL	0.40–4.50	—
Vitamin D, 25-OH	28 ng/mL	30–100	L
Mildly deficient; start D3 2000 IU/day			
Ferritin	24 ng/mL	13–150 (female)	—

FOLLOW-UP & NEXT STEPS

- Return in 4 weeks for repeat BMP (potassium and ALT recheck).
- Return in 3 months for repeat prolactin level.
- Schedule DEXA bone density scan within 60 days.
- Begin Vitamin D3 2000 IU daily — available OTC.
- Continue avoiding NSAIDs and excessive alcohol while on spironolactone.
- Next routine 6-month HRT monitoring visit: November 2026.
- MyChart message or call (503) 555-0187 with any concerns before next visit.

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